جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : داليا فاروق عبدالواحد سعد
المحافظة : القليوبيه **الحي / المدينة :** طوخ **العنوان :** العمرانية الجيزة
رقم البطاقة : 27808271402006 **رقم التليفون :** 01220462002 **تاريخ تسجيل الإستمارة :** 21-10-2026
العمر(تاريخ الميلاد) : 27-08-1978 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: داليا فاروق عبدالواحد سعد
National ID: 27808271402006

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

٨٥

National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

أسبوع مصر صحة

MDT FORM 29/7/2025 G

ne	دانيا فاروق عبدالواحد	MRN	1495/25
	46y.	ID	
nopausal status	Pre...	Phone	٠١٢٢٤٦٢٠٠٢
S SCORE		Marital status	Unm. Muh...
l		Family history	
morbidity		REASON OF MDT	New

٢٧٨-٨٢٧١٤٠٠٢

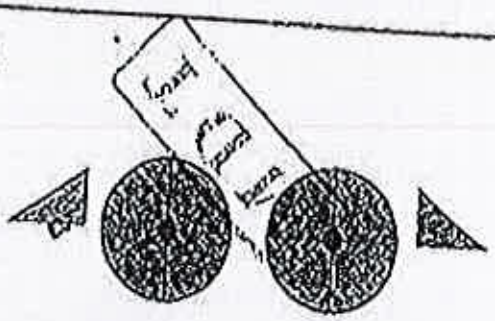
Clinical Data

Mx Rt IDC & ILC Gr II

	PR:	HER 2:	Ki67:
/18	0/8	3+	25%

mammography: Rt UOQ susp. mass + nipple ulcer Paget's disease

ts Workup: CT, Bone scan: Pending



T4 N1 M0

Rt nipple ulcer + skin edema & peau d'orange

commendations: For possible Rt MRM

Dr. Hala 27/7/2025

Dr. Amira Essa 27/7/2025

T OUTCOME & PLAN

1. Start Neoadj CT TC(T1P) x6 cycles → plan to
2. then Rt MRM

SIGNATURE

DICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: داليا فاروق عبد الواحد سعد	
MR. : 4767	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presented for the first cycle (C1) of Kadcyla 260 mg after laboratory work and an echocardiogram performed on 2-2026, which demonstrated a left ventricular ejection fraction of 69 %.

The patient was diagnosed with right-sided breast cancer, clinical stage T4 N1, with pathology indicating invasive ductal carcinoma, estrogen receptor (ER) 0/8, progesterone receptor (PR) 0/8, HER2 3+, and Ki-67 23 %. A multidisciplinary team recommended trastuzumab-deruxtecan (Kadcyla) and scheduled the first administration on 7-2-2026; pathology from the right mastectomy (MRM) was pending immunohistochemistry (IHC).

For the third cycle (C3) of Kadcyla 260 mg, the patient awaited IHC results on the mastectomy specimen; the most recent ejection fraction measured on 11-5-2026 was 70 %.

The patient completed the fourth cycle (C4) of Kadcyla 260 mg; cycles C5 and C6 were prepared and available in the pharmacy.

For the fifth cycle (C5) of Kadcyla, laboratory results were accepted and an echocardiogram was ordered prior to proceeding with the sixth cycle (C6), which was also stocked in the pharmacy.

Immunohistochemistry on the mastectomy specimen confirmed ER 0/8, PR 0/8, and an estrogen fraction of 68 % on 10-8-2026; the patient received the sixth cycle (C6) of Kadcyla 260 mg.

Immunohistochemistry on the mastectomy specimen showed ER 0/8, PR 0/8, HER2 + 3, and an estrogen fraction of 68 % on 10-8-2026; the patient received the seventh cycle (C7) of Kadcyla 260 mg, with cycles C8 and C9 prepared in the pharmacy. The patient has now completed seven cycles of Kadcyla.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

nt Name	: الأستاذه / دانيا فاروق عبد الواحد سعد	Request No.	: 13007964
nt No.	: 27808271402006	Request Date	: 26/05/2025
No.	: SR-747/25	Verification Date	: 02/06/2025
	: Female	Financial Category	: جوستاف روسي - تغطية الدولة
	: 46 Y	Department	: Out Patient
rreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

ical Data: Right breast retro-areolar mass.
ure of Specimen: Slides Revision (Tru-cut biopsy).
ss Examination:
ne paraffin block labeled as (953453) and five slides were received.
roscopic Examination:
stological type: Mixed invasive duct & lobular carcinoma (Predominant ductal
ponent and minor lobular component).
rade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+2).
CIS: Minimally detected.
ngioinvasion: Not detected.
erineural invasion: Not detected.
>T:
R: Negative nuclear reaction in tumor cells (score 0/8).
R: Negative nuclear reaction in tumor cells (score 0/8).
er2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
i67: 25% nuclear reaction of tumor cells.

agnosis:
ght breast retro-areolar mass, Slides revision (Tru-cut biopsy):
fixed invasive duct & invasive lobular carcinoma, Grade II (Predominant ductal element &
nor lobular element), with minimal ductal orcinoma insitu.
T:
ER: Negative (score 0/3).
PR: Negative (score 0/8).
Her2: Positive (score 3+).
Ki67: Positive in 25% of tumor cells.

Scoring system: "ER, PR"

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

ent Name	: الأستاذة / داليا فاروق عبد الواحد سعد	Request No.	: 13007964
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ı No.	: SR-747/25	Verification Date	: 02/06/2025
:	: Female	Financial Category	: جوستف روسي - تقة الدولة
)	: 46 Y	Department	: Out Patient
erreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

:CO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no
ıining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and
%
d 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1,
ek, 2, intermediate; and 3, strong). Total score from 0 to 8.
coring system: "Her2"
ference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no
aining observed or membrane staining that is incomplete and is faint/barely perceptible
d within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane
aining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
C 2+: based on circumferential membrane staining that is incomplete and/or
eak/moderate and within >10% of
e invasive tumor cells or complete and circumferential membrane staining that is intense
id
thin < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane
aining that is complete, intense > 10% of tumor cells.

ignature
)r. Walaa Ghoneim.
)r. Heba Hassan.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadeem - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : داليا فاروق عبد الواحد سعد
Age : 46 Y : 8 M : 1 D
Analysis ID : 953455
Gender : Female
Referred By Prof. Dr. : جمال ابو العلا
Clinical Diagnosis : Right breast retroareolar mass
Nature of Specimen : Block (US guided tru cut biopsy, Labelled as 953453)
Receiving Date : 2025-05-11
Patient ID : 707845

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .
The sections were treated against:
- Estrogen receptors -Progesterone receptors - Her 2 - Ki67

Results :

Sections treated for Estrogen receptors revealed NEGATIVE nuclear staining.
Sections treated for Progesterone receptors revealed NEGATIVE nuclear staining.
Sections treated for HER2 revealed STRONG CONTINUOUS CELL MEMBRANE STAINING (CHICKEN WIRE) IN MORE THAN 10% OF TUMOR CELLS
18% of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE
NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8)
NEGATIVE FOR PROGESTERONE RECEPTORS (SCORE 0/ 8)
HER 2 POSITIVE, SCORE 3+
THE PROLIFERATION INDEX OF Ki67 IS 18%.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

DR ELIA ANIS LABS

This report is electronically signed , any alteration or modification would render it void .

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٢٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name: Dalia Farouk Abd Al Wahed	Date : 09/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 47 Year(s)
Medical ID: 1495/2025	Pat. ID: 100419

POST -CONTRAST CT SCAN OF THE CHEST , ABDOMEN AND PELVIS : -

Findings:

- Left thyroid lobe nodule 4.1 x3.7 cm .
- Clear both lung fields
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average size cardiac size and shape. No pericardial effusion detected.
- Normal appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Average size liver showing homogenous density . No C.T. evidence of hepatic focal lesions.
- * Average size spleen , no focal lesions .
- *Average size gall bladder , no gall stones.
- *No dilated intra or extra-hepatic bile ducts .
- *Patent main portal vein trunk and both branches.
- *Urinary bladder shows no masses or stones.
- *Normal C.T. appearance of the pancreas , adrenal glands, both kidneys ,bowel loops & other pelvic organs.
- *No retroperitoneal lymphadenopathy .
- *No ascites .
- The examined bony skeleton shown dorsolumbar scoiloisis with apex at L1/L2 level to the right .
- No worrisome lytic or sclerotic bony lesions.

Opinion:

- *Unremarkable study apart from left thyroid nodule for neck US.*

Dr/Reham Khodr...MD

CT Scan
[[DOCTYPE:CT_SCAN]]

ECHO REPORT

Name:

Date: ٢٤ / 9 / 2025

EF	70 %	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	4.4	3.5-5.8cm	AO.R	2.3	2-3.7cm
LVESD		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3cm(sup)
SWT	0.6	0.6-1.1cm	AO.CUSP		1.5-2.6cm

Patient is Tachycardic

LV dimensions ——— normal

LV wall thickness ——— normal

LV diastolic function ——— normal

SWMA ——— no SWMA

LA and Ao root dimensions ——— normal

Mitral Valve ——— Mild MR

Aortic valve ——— normal

Tricuspid valve ——— Mild TR normal PASP

Pulmonary valve ——— normal

Right side and systolic function ——— normal

Intracardiac masses and effusion ——— no

Dr /

Patient Name: Dalia Farouk Abd El-Wahed

Patient ID: 750832
Exam Date: 21/7/2025

^{99m}Tc-MDP whole body bone scan

Desired information:

- *Skeletal surveying*

Technique:

- *Anterior and posterior Delayed Whole body bone scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:*

Report:

Delayed static phase:

- *Degenerative changes are noticed in the dorsal &lumbar vertebrae, more at lower dorsal.*
- *Mild Arthritic changes at both shoulder and knee joints.*
- *There is no hot spot that account for osseous deposits could be detected allover the surveyed skeleton.*
- *Both kidneys are visualized.*

CONCLUSION:

- *The surveyed skeleton is free from definite osseous deposits or serious bone lesions.*
- *Spondylo- degenerative changes at the spine.*

Best regards;
Dr. Huda Fathy M.D.

Bone Scan
[[DOCTYPE:BONE_SCAN]]

alfa
SCAN

Name: Dalia Farouk Abd Al Waked ID: 4421951
Referred by: Prof. Wael Tobar Date: 06/05/2025

Thanks for reference

Bilateral digital mammography , high resolution breast sonography & Elastography revealed :

- Scattered mixed densities of glandular condensation are seen involving bo breasts.
- Right nipple LIQ suspicious pleomorphic microcalcifications and US detect solid hypoechoic illdefined mass in the 4:00 location midzonal measures 15x m , hard by Elastography and there is associated right nipple ulcer .
- left UOQ lumpectomy scar and left UOQ benign calcification.
- No left breast solid or cystic masses.
- Enlarged right axillary LN measures 24x12 mm on diameters
- Small sized likely non-specific left axillary lymph nodes.
- Normal skin thickness & contour of both breasts.

OPINION :

Bilateral mild mammary fibroadenosis & left UOQ unremarkable lumpectomy scar. BIRADS 2.

Right LIQ suspicious mass and nipple ulcer and ipsilateral lymph no enlargement likely Paget's disease & NOS carcinoma ; BIRADS 5 , biopsy recommended.

Kindly accept my best regar
Dr. Naglaa Abdel Razek, M.

Defining BIRADS classificatior of breast lesions according to the American College of Radiology:

Category	Suspected Cancer Risk
BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	2 %
BIRADS 4 : Probably malignant.	50 %
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	

Mammogram
[[DOCTYPE:MAMMOGRAM]]